

I can't stand the pain, Doc!

**(Pain, pathways and
private insurance)**

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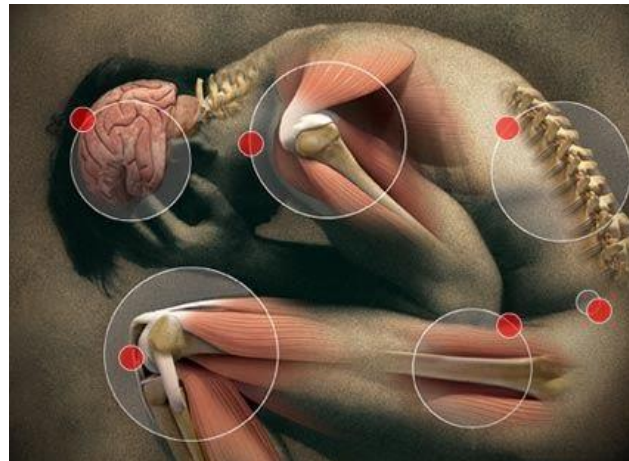
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Introduction

Pain is defined as-

An unpleasant sensory and emotional experience associated with actual or potential tissue damage, or described in terms of such damage

Source: International Association for Study of Pain, 2011







Some facts and figures

- ▶ 49% of UK adult population report back pain lasting at least 24 hours in the previous year
- ▶ 40% attend GP reporting back pain and 10% attend a chiropractor or osteopath
- ▶ 80% of UK adults will experience back pain at some point in their lives
- ▶ In 85% of cases, no cause of back pain can be identified
- ▶ Most episodes are self-limiting and 90% recover within 6 weeks- however 7% develop chronic pain
- ▶ Costs to NHS reported to be in excess of £1,000,000,000 (one billion) with private sector healthcare costs in excess of £500,000,000 (500 million)
- ▶ Health and safety Executive (HSE) estimates £600,000,000 cost to employers or around 1-2% of Gross Domestic Product and with 5,000,000 working days lost (in 2003-04)
- ▶ 1% of workforce absent with back pain on any one day
- ▶ Back pain is the second most common reason (after mental health) for long term absence due to sickness (commonest in manual labour occupations)

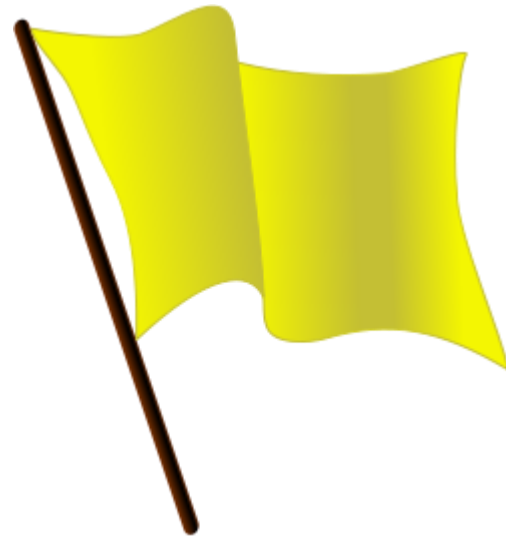
GP scenario



Please take a minute or so to look at
the scenario
How should I proceed?

Look at handout now

'Red Flags' and 'Yellow Flags'



Red Flags and Yellow Flags

Suggest serious pathology

- ▶ Urinary retention or incontinence
- ▶ Age of onset over 55 or under 20
- ▶ Weight loss, fever- 'systemic illness'
- ▶ Morning stiffness
- ▶ Progressive severe pain
- ▶ History of cancer
- ▶ Intravenous drug use
- ▶ Prolonged steroid use

Psychosocial factors

- ▶ Psychological
 - Attitudes and beliefs
 - Distress and depression
 - Excessive adoption of sick role
- ▶ Social
 - Family- marital strife, financial problems
 - Medico-legal proceedings
- ▶ Work
 - Physical demands
 - Job Satisfaction
 - Poor health record at work

Management of back pain

Categorised into 3 groups

- ▶ Simple back pain/ low back pain
- ▶ Nerve root pain
- ▶ Possible serious spinal pathology (Red flags)

Simple Back Pain

- ▶ Age 20-55 years
- ▶ Pain in lumbo-sacral area, buttocks thighs
- ▶ 'Mechanical' pain (pain on specific postures or movements)
- ▶ Patient is well



Nerve Root pain



- ▶ Unilateral leg pain greater than back pain
- ▶ Radiating to foot/ toes
- ▶ Sensory disturbance in same distribution
- ▶ Reduced straight leg raise which reproduces leg pain
- ▶ Reduced power and reflexes in expected distribution

Possible serious spinal pathology



Management of back pain

- ▶ Psychological
- ▶ Drug treatment
- ▶ Other therapies

Psychological management

- ▶ Encourage positive attitude towards recovery
- ▶ Reassurance
- ▶ Encouragement to keep active and remain at work
- ▶ Avoid bed rest



Drug treatment



- ▶ Regular, not 'as required' analgesics
- ▶ Begin with paracetamol
- ▶ Add anti-inflammatory, if inadequate
- ▶ Try weak opiate combined with paracetamol (Co-codamol or co-dydramol)
- ▶ Consider muscle relaxant

Other therapies



Other therapies

- ▶ Manipulation- strong evidence of better short-term improvement in pain and activity and higher patient satisfaction. Moderate evidence of very low risk in trained hands
- ▶ Back Exercises- moderate evidence that can improve pain and function in chronic low back pain. Strong evidence that no significant improvement in acute low back pain
- ▶ Evidence for TENS, back schools, local injections, shoe lifts inconclusive
- ▶ No evidence for use of back supports or corsets, acupuncture, traction, ultrasound, heat, ice, diathermy or massage
- ▶ Evidence against use of plaster jackets, steroids or strong opiates or benzodiazepines beyond 2 weeks



Who and when to refer



- ▶ Cauda equina
- ▶ Red Flags
- ▶ Nerve root pain not resolving after 4 weeks

How should I manage Carol Smith?

- ▶ You advise Carol about work and physical activity and provide an advice leaflet explaining the simple messages around back pain and how to protect the back when lifting and doing heavy work.
- ▶ You recommend simple but regular analgesics, especially at night
- ▶ Some elements of Carol's history raise your concerns about a possible poor prognosis for improvement and associated increased risk for time off work
- ▶ You discuss with Carol the need for a follow-up appointment and the possibility of a referral to a physiotherapist. You also suggest that she discusses her concerns about the lack of help for lifting with her supervisor

Yellow Flags

Psychosocial factors

- ▶ Psychological
 - Attitudes and beliefs
 - Distress and depression
 - Excessive adoption of sick role
- ▶ Social
 - Family- marital strife, financial problems
 - Medico-legal proceedings
- ▶ Work
 - Physical demands
 - Job Satisfaction
 - Poor health record at work

Concerns from Carol's history

- ▶ 'She offers the information that staff illness and absence rates in her workplace have been higher than usual in recent months, with change of personnel in the senior management of the home. 'It's not like it used to be – it's more stressed – there are not the people around to help with lifting and moving like before'
 - (Job satisfaction?)
- ▶ 'She expresses her concern that she might be developing a long-term problem which will make her work difficult'
 - (Attitude and beliefs?)
- ▶ 'She tells you that a friend of hers had a scan and a special injection for her back pain which really helped. She wonders whether you think she should be referred to a specialist for this?'
 - (Attitude and beliefs? Excessive adaption of sick role?)
- ▶ 'at night, has started to feel a bit low, and she gets more tired than before towards the end of the day'
 - (Distress and depression?)

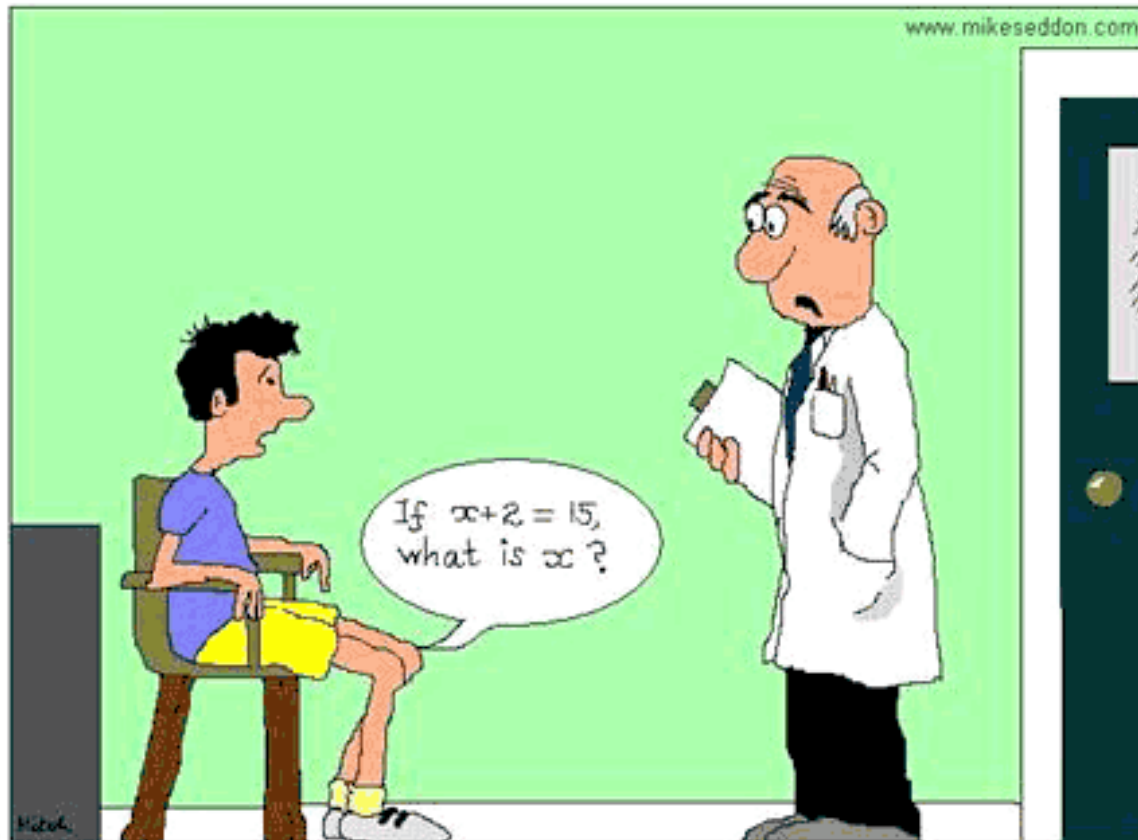
What happens next?



- ▶ Most (90%) of episodes of low back pain settle within 6 weeks (50% in nerve root pain)
- ▶ If she doesn't improve, consider;
 - Tricyclic antidepressants (amitryptiline)
 - Strong opiates (for short-term use)
 - Referral for structured exercise programme or manual therapy
 - Referral for combined physical and psychological programme which includes cognitive behavioural approach and exercise (often by means of referral to local pain service)
 - Course of acupuncture
 - Surgical referral if continued severe pain despite optimal care and treatment of psychological distress

Vertical Streaming

By Mike Seddon



"It's my knee, Doctor. It's still giving me problems."